

CITYCARE GENERAL HOSPITAL

# WARD ALLOCATION POLICY

Inpatient Bed Assignment, Prioritisation & Escalation Framework

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|-------------------|-------------------------------------------------------------------|
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## 1. PURPOSE, SCOPE & GUIDING PRINCIPLES

This policy establishes a fair, clinically driven, and operationally efficient framework for the allocation of inpatient ward beds at CityHospital. It applies to all patients admitted via the Emergency Department, planned elective pathways, inter-hospital transfers, direct specialist admissions, and post-operative recovery. The policy ensures that ward allocation decisions are transparent, equitable, and always based on clinical need rather than administrative convenience.

### 1.1 Policy Objectives

- Ensure every patient is allocated the most clinically appropriate bed based on their care needs and acuity.
- Optimise hospital-wide bed occupancy, minimising unnecessary delays and bed shortfalls.
- Prevent misallocation of specialist, isolation, or high-dependency beds that are required by other patients.
- Promote patient dignity by maintaining gender-appropriate ward allocation wherever possible.
- Define clear escalation steps when demand exceeds available bed capacity.
- Reduce average time from admission decision to patient in bed to 60 minutes or less.
- Comply with national standards for inpatient accommodation, fire safety, and infection prevention.

### 1.2 Guiding Principles

- **Clinical need first:** Acuity and clinical requirement always override administrative, financial, or logistical factors.
- **Equity:** No patient shall receive preferential or inferior allocation on the basis of gender, ethnicity, religion, age, or ability to pay.
- **Dignity & privacy:** Single-sex bay allocation is the default standard; mixed-sex placement is an exception requiring documentation and consent.
- **Transparency:** All allocation decisions are documented in real time in the Bed Management System (BMS).
- **Safety:** Infection control, falls risk, and clinical observation needs are factored into every allocation decision.

**NOTE:** This policy must be read alongside SOP-ADM-002 (Patient Admission SOP) and SOP-IPC-015 (Infection Prevention & Control SOP). In the event of conflict, the most clinically protective standard applies.

## 2. DEFINITIONS & ABBREVIATIONS

| Term / Abbreviation     | Definition                                                                                                                                               |
|-------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------|
| BMS                     | Bed Management System — the electronic real-time platform tracking all bed status, occupancy, and allocation across the hospital.                        |
| Bed Manager             | The designated operational lead responsible for real-time bed allocation, coordination, and escalation across all wards.                                 |
| Elective Admission      | A pre-planned, scheduled inpatient admission for a specific procedure or treatment episode.                                                              |
| Emergency Admission     | An unplanned admission arising from the Emergency Department or urgent clinical referral.                                                                |
| Outlier Bed             | A bed used by a patient in a ward outside their primary clinical specialty due to capacity constraints.                                                  |
| Cohort Ward             | A dedicated ward or bay used to group patients with the same diagnosis or infection to limit cross-transmission.                                         |
| Isolation Room          | A single-occupancy room used to separate patients with confirmed or suspected communicable disease.                                                      |
| Surge Capacity          | Additional beds or clinical spaces activated when standard bed occupancy is critically exceeded.                                                         |
| Discharge Lounge        | A designated area where clinically fit-for-discharge patients wait for transport or medication, freeing the inpatient bed earlier.                       |
| Step-Down Bed           | A bed in an intermediate care or general ward for a patient transitioning from ICU or HDU.                                                               |
| ALOS                    | Average Length of Stay — a key metric used to forecast bed demand and plan elective admissions.                                                          |
| Mixed-Sex Accommodation | The placement of male and female patients in the same bay or room; a deviation from standard that requires documented justification and patient consent. |
| HCA                     | Healthcare Assistant — clinical support staff who assist with ward rounds and bed preparation.                                                           |
| Portering Team          | Hospital staff responsible for the safe physical transfer of patients between departments and wards.                                                     |

### 3. ROLES & RESPONSIBILITIES

| Role                                  | Department          | Ward Allocation Responsibilities                                                                                                           |
|---------------------------------------|---------------------|--------------------------------------------------------------------------------------------------------------------------------------------|
| Bed Manager (24/7)                    | Bed Management Unit | Real-time BMS oversight, bed allocation decisions, escalation coordination, surge management, daily bed state reporting to Site Management |
| Site Manager / Hospital at Night Lead | Site Management     | Out-of-hours escalation authority, cross-directorate bed decisions, major incident bed release                                             |
| Ward Manager / Charge Nurse           | All Wards           | Notify BMS of available beds immediately post-discharge, receive incoming patients, manage ward-level capacity                             |
| Admitting Physician / Specialist      | All Specialties     | Specify clinical ward requirements at point of admission decision; communicate acuity and isolation needs                                  |
| Emergency Department Consultant       | Emergency Dept.     | Notify Bed Manager of pending admissions; SBAR handover; escalate boarding patients beyond 4 hours                                         |
| Infection Control Nurse               | IPC Team            | Advise on isolation room need, cohort ward allocation, and infection risk prioritisation                                                   |
| Discharge Coordinator                 | Discharge Team      | Proactively free beds by coordinating early and planned discharges; manage discharge lounge flow                                           |
| Portering Team                        | Operations          | Transfer patients to allocated wards within 30 minutes of notification; log all transfers in BMS                                           |
| Housekeeping / Domestic Services      | Facilities          | Turn beds over within 45 minutes of discharge; notify BMS when bed is clean and ready                                                      |
| Finance / Insurance Team              | Finance             | Not involved in clinical allocation decisions; any financial pressure on bed allocation escalated to CMO immediately                       |

## 4. WARD ALLOCATION CRITERIA & CLINICAL PRIORITY FRAMEWORK

Ward allocation follows a clinical priority hierarchy. The Bed Manager must always work through the priority tiers in order, allocating the most appropriate bed for the highest clinical need first.

### 4.1 Clinical Priority Tiers for Bed Allocation

| Priority         | Patient Category                                                      | Allocation Target                                                      | Time to Bed                                            |
|------------------|-----------------------------------------------------------------------|------------------------------------------------------------------------|--------------------------------------------------------|
| P1 — Critical    | ICU/HDU step-down; post-resuscitation; post-operative high-risk       | ICU, HDU, or monitored step-down bed with telemetry                    | Immediate — within 15 minutes                          |
| P2 — Urgent      | ED Level 2 admit; acute medical emergency; major surgery post-op      | Specialty ward (surgical, medical, cardiac) closest match              | Within 30 minutes of admission decision                |
| P3 — Semi-Urgent | ED Level 3 admit; acute medical — stable; elective surgery post-op    | Appropriate specialty ward; outlier only if no specialty bed available | Within 60 minutes                                      |
| P4 — Planned     | Elective admission; pre-booked surgical or medical day case overnight | Pre-allocated ward bed confirmed 24 hrs in advance                     | As per pre-admission booking                           |
| P5 — Transfer    | Inter-hospital transfer; repatriation admission                       | Specialty-matched bed; isolation if infection risk                     | Bed confirmed before transfer departs sending hospital |

### 4.2 Clinical Specialty Matching

- Patients must be allocated to a bed within their admitting clinical specialty as the first preference.
- Cardiology, oncology, paediatrics, obstetrics, and infectious disease patients must not be placed as outliers on unrelated wards except in formally declared surge situations.
- Outlier placement requires: written Bed Manager authorisation, admitting consultant agreement, and documentation in BMS with clinical rationale.
- Outlier patients must be reviewed daily by their admitting specialty team; repatriation to the correct specialty ward actioned within 24 hours of a bed becoming available.

### 4.3 Gender-Appropriate Allocation

- All ward bays must be single-sex; male and female patients must not share sleeping, toilet, or bathing facilities.
- Mixed-sex accommodation is only permitted as a last resort in a clinical emergency; patient or guardian must be informed and consent documented.
- Mixed-sex placements must be reported to the Ward Manager and Bed Manager within 1 hour; resolved within 8 hours wherever clinically safe.
- Mixed-sex accommodation incidents tracked monthly and reported to the clinical governance board.

## 5. STEP-BY-STEP WARD ALLOCATION WORKFLOW

### 5.1 Emergency & Unplanned Admissions

- Step 1.** ED physician makes admission decision; documents in EMR with specialty, acuity level, isolation requirements, and clinical priority (P1–P5).
- Step 2.** ED nurse notifies Bed Manager via BMS portal or dedicated Bed Manager telephone line; SBAR handover completed.
- Step 3.** Bed Manager reviews live BMS dashboard; identifies best available bed matching specialty and priority criteria.
- Step 4.** Ward Manager of the receiving ward notified by phone and BMS alert; confirms readiness to receive patient.
- Step 5.** Bed Manager confirms allocation in BMS; sends electronic notification to ED, portering team, and receiving ward simultaneously.
- Step 6.** Porter despatched to collect patient within 15 minutes of allocation confirmation; transport time logged in BMS.
- Step 7.** Receiving ward nurse acknowledges patient arrival in BMS; ED bed released and flagged for housekeeping.
- Step 8.** If no suitable bed available: P1–P2 patients remain in ED monitored area; Site Manager notified immediately; escalation protocol activated.

### 5.2 Elective & Pre-Planned Admissions

- Step 1.** Pre-admission office confirms surgical or medical list 48 hours in advance; bed requests submitted to Bed Manager by 16:00 the day before admission.
- Step 2.** Bed Manager pre-allocates beds for elective patients based on specialty, procedure type, anticipated length of stay, and gender.
- Step 3.** Any changes to elective list (cancellations, additions) communicated to Bed Manager by 08:00 on the day of surgery.
- Step 4.** Patient arrives at Admissions; Bed Manager confirms pre-allocation is still valid in BMS.
- Step 5.** Admissions nurse notifies Bed Manager of patient arrival; porter called to transfer patient to pre-allocated bed.
- Step 6.** Post-operative patients: theatre coordinator notifies Bed Manager 30 minutes before patient expected in PACU to confirm ward bed availability.

## 6. SPECIALIST & COHORT WARD ALLOCATION

### 6.1 Specialist Ward Allocation Requirements

| Specialty                           | Mandatory Ward / Area                       | Exception Conditions                                                                        |
|-------------------------------------|---------------------------------------------|---------------------------------------------------------------------------------------------|
| Paediatrics (under 16 yrs)          | Paediatric Ward only                        | Neonatal ICU for neonates; PICU for critically ill children                                 |
| Obstetrics & Maternity              | Maternity Unit only                         | HDU or ICU if maternal critical illness (with obstetric review)                             |
| Oncology / Haematology              | Oncology Ward (isolated bay if neutropenic) | ICU if life-threatening; reverse barrier nursing applies                                    |
| Infectious Disease / Confirmed MDRO | Isolation room or Infectious Disease ward   | Cohort bay if single isolation rooms unavailable (IPC approval required)                    |
| Cardiac / Telemetry                 | Cardiac ward or monitored bed               | ICU/HDU if haemodynamically unstable                                                        |
| Acute Stroke                        | Stroke Unit within 4 hours of stroke onset  | ICU if requiring ventilation; repatriated to stroke unit as soon as safe                    |
| Burns                               | Burns Unit (or regional centre transfer)    | HDU for major burns >20% body surface area pending transfer                                 |
| Mental Health (admitted under MHA)  | Psychiatric ward or PICU                    | General ward only under 1:1 observation if psychiatric bed unavailable and medically unwell |

### 6.2 Cohort Ward Activation

- Cohort wards are activated by the IPC Lead and CMO during an outbreak or when isolation room capacity is exhausted.
- A nominated ward or bay is designated as the cohort area; only confirmed cases of the same organism placed together.
- Cohort ward staff are dedicated to that area during the outbreak period; no cross-working with non-cohort wards.
- Cohort ward signage, PPE stations, and restricted access implemented within 2 hours of activation.
- Cohort ward deactivated by IPC Lead only after all patients are clear and thorough terminal clean is complete.



## 7. INFECTION CONTROL & ISOLATION ROOM ALLOCATION

Isolation room allocation is a clinical safety priority. The Infection Control Nurse and Bed Manager jointly govern isolation room availability. Isolation rooms must never be used for administrative convenience or elective patient preference.

### 7.1 Isolation Room Prioritisation Order

| Priority       | Indication                                                                                         | Type of Isolation                                    |
|----------------|----------------------------------------------------------------------------------------------------|------------------------------------------------------|
| 1 — Immediate  | Active pulmonary TB, measles, chickenpox, suspected haemorrhagic fever, novel respiratory pathogen | Negative pressure isolation room; full airborne PPE  |
| 2 — Urgent     | Confirmed MRSA bacteraemia, confirmed C. difficile (diarrhoea), active norovirus                   | Standard isolation room; contact precautions         |
| 3 — Standard   | Suspected MDRO (ESBL, CRE pending confirmation), open wound with resistant organism                | Standard isolation room; contact precautions         |
| 4 — Protective | Neutropenic patients (ANC below 0.5), post-transplant, on high-dose immunosuppression              | Protective isolation; positive pressure if available |

### 7.2 Isolation Room Shortage Protocol

- If no isolation room is available: IPC Nurse notified immediately; risk-stratified decision made jointly by IPC Nurse and Bed Manager.
- Highest-risk patient (airborne transmission) takes absolute priority for the next available room.
- Interim measures while awaiting isolation room: patient cohorted at end of bay, curtains drawn, dedicated staff, PPE signage applied.
- Bed Manager accelerates discharge of any patient currently in an isolation room who no longer clinically requires it.
- IPC Lead informed of all isolation room shortages; escalated to CMO if resolution not achieved within 4 hours.

**WARNING: Patients with suspected or confirmed airborne infectious disease (TB, measles, novel respiratory pathogen) MUST be placed in a negative pressure room or remain in the resuscitation bay with door closed until a suitable room is available. DO NOT place in an open bay under any circumstances.**

## 8. ESCALATION, SURGE & OVERFLOW BED MANAGEMENT

### 8.1 Escalation Trigger Levels

| Trigger Level           | Occupancy / Condition           | Actions Required                                                                                                                               |
|-------------------------|---------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------|
| Level 1 — Green         | Below 85% occupied              | Normal operations; routine bed management; daily discharge planning continues                                                                  |
| Level 2 — Amber         | 85–94% occupied                 | Bed Manager reviews all anticipated discharges; discharge lounge opened; elective admissions reviewed; ward managers briefed                   |
| Level 3 — Red           | 95–99% occupied                 | Site Manager activated; non-urgent elective admissions deferred; day surgery unit opened as overflow; all available discharge routes expedited |
| Level 4 — Black (Surge) | 100% occupied or Major Incident | CMO and executive team notified; Major Incident Plan activated; day case and step-down areas converted; inter-hospital transfer considered     |

### 8.2 Surge Bed Activation Protocol

**Step 1.** Bed Manager declares Level 3 or 4; Site Manager and CMO informed immediately; BMS status set to SURGE.

**Step 2.** Discharge Coordinator fast-tracks all patients meeting discharge criteria; discharge lounge capacity expanded.

**Step 3.** Day surgery unit activated as additional inpatient overflow; theatre schedule reviewed for deferrable cases.

**Step 4.** All ward managers contacted; any boardable beds (side rooms used for non-clinical reasons) released immediately.

**Step 5.** Ambulance divert request submitted to regional ambulance service if ED boarding exceeds 10 patients.

**Step 6.** Inter-hospital transfer pathway activated for clinically stable patients who can be safely transferred to partner hospitals.

**Step 7.** Hourly BMS status updates issued to all clinical areas until occupancy returns below 90%.

## 9. BED TURNAROUND, DISCHARGE PLANNING & HOUSEKEEPING

### 9.1 Bed Turnaround Standards

| Stage                                                                      | Responsible Team        | Time Target                               |
|----------------------------------------------------------------------------|-------------------------|-------------------------------------------|
| Physician enters discharge order in EMR                                    | Attending Physician     | By 09:30 for planned discharges           |
| Ward nurse completes discharge process (education, medications, checklist) | Ward Nursing Team       | Within 90 minutes of discharge order      |
| Patient physically leaves the ward                                         | Patient / Porter / Ward | By 11:00 for planned discharges           |
| Ward nurse marks bed as 'vacated' in BMS                                   | Ward Nurse              | Immediately upon patient departure        |
| Housekeeping team notified and commences bed clean                         | Housekeeping / Domestic | Within 10 minutes of BMS 'vacated' status |
| Bed cleaned, changed, and marked 'ready' in BMS                            | Housekeeping Supervisor | Within 45 minutes of 'vacated' status     |
| Bed Manager allocates 'ready' bed to next waiting patient                  | Bed Manager             | Immediately upon 'ready' status           |

### 9.2 Discharge Lounge Policy

- Patients who are clinically fit for discharge but awaiting transport, medication, or family collection must be transferred to the Discharge Lounge to free the inpatient bed.
- Discharge Lounge capacity: up to 20 ambulatory patients; nursing supervision provided.
- Patients requiring ongoing IV fluids, oxygen, or continuous monitoring are not eligible for Discharge Lounge.
- Discharge Lounge open 08:00–20:00; out-of-hours patients remain on ward if discharge expected after 20:00.

### 9.3 Bed Blocking Prevention

- Estimated Discharge Date (EDD) set at admission; reviewed at every ward round; updated in BMS.
- Beds occupied beyond EDD without clinical justification flagged to Bed Manager and Ward Manager daily.
- Social work referral for patients delayed for social or care package reasons made within 24 hours of identification.
- Weekly 'long-stay patient' review (LOS exceeding 14 days): multidisciplinary review to identify and remove all barriers to discharge.

## 10. EXCEPTIONS, EQUITY SAFEGUARDS & BEST PRACTICES

### 10.1 Managing Exceptions

- Any allocation that deviates from this policy (outlier placement, mixed-sex bay, non-specialty ward) must be documented in BMS with the clinical or operational reason within 1 hour.
- Exceptions driven by clinical emergency (e.g., no isolation room available for airborne pathogen) do not require prior authorisation but must be documented immediately after the event.
- Repeated exceptions by any ward or department trigger a review by the Bed Management Committee within 30 days.
- Financial or insurance-related ward allocation requests are never acted upon; any such pressure reported to the CMO as a governance concern.

### 10.2 Equity Safeguards

- Allocation decisions are regularly audited for unintentional bias; demographic data reviewed quarterly by the Clinical Governance Committee.
- Patients with disabilities or special communication needs flagged in BMS; allocation considers proximity to accessible facilities.
- Patients from vulnerable groups (elderly, learning disability, mental health) allocated to bays with enhanced staff supervision where possible.
- Complaints related to ward allocation handled via the Patient Relations team within 7 working days; themes reviewed monthly.

### 10.3 Audit & Performance Monitoring

| KPI                                                    | Target                                | Review Frequency |
|--------------------------------------------------------|---------------------------------------|------------------|
| Time from admission decision to patient in bed (P1–P2) | Less than 30 minutes                  | Daily            |
| Time from admission decision to patient in bed (P3)    | Less than 60 minutes                  | Daily            |
| Bed turnaround time (vacated to ready)                 | Less than 45 minutes                  | Daily            |
| Outlier patients repatriated within 24 hours           | Greater than 90%                      | Weekly           |
| Mixed-sex accommodation incidents                      | Zero (target) — tracked               | Monthly          |
| Elective admission bed confirmed 24 hrs in advance     | Greater than 95%                      | Weekly           |
| Discharge lounge utilisation rate                      | Greater than 70% of eligible patients | Monthly          |

### 10.4 Best Practices

- Morning bed huddle at 08:30 daily: Bed Manager, Site Manager, Ward Managers, and Discharge Coordinator attend; all anticipated discharges, admissions, and blockers discussed.
- Evening bed review at 17:00: Bed Manager and Site Manager review overnight demand; pre-position resources for anticipated night admissions.
- Proactive patient flow: identify patients likely to be discharged the following day at each afternoon ward round; notify Bed Manager to plan allocation for incoming admissions.

- BMS data reviewed weekly: trends in allocation delays, outlier rates, and bed blocking shared with all ward managers and department heads.
- Annual ward allocation policy review: policy updated based on audit findings, staff feedback, and changes to hospital capacity or specialty configuration.